WEEKLY EPIDEMIOLOGICAL REPORT

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WEEKLY EPIDEMIOLOGICAL REPORT SRI LANKA 2026

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Strengthening Yellow Fever Preparedness in Sri Lanka: A Public Health Perspective

Current context

From late 2025 through mid-2026, the global Yellow Fever situation has been marked by transmission in the WHO African Region (AFR) and an increase in activity in the WHO Region of the Americas (AMR). Together, these events involve 13 of the 40 countries classified as high risk under the EYE (Eliminate Yellow Fever Epidemics) strategy, reflecting a period of heightened epidemiological activity. The rapid risk assessment revealed "Low" overall public health risk globally and moderate risk for WHO AFR & WHO AMR.

Introduction

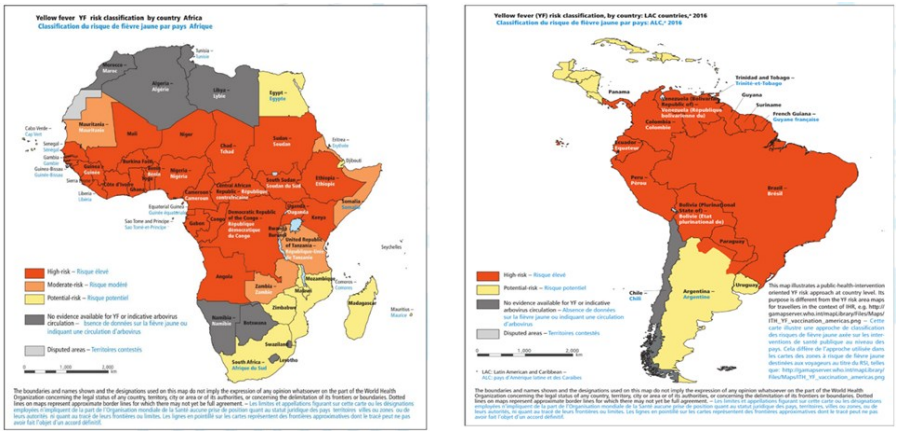
Yellow fever is a vaccine-preventable, mosquito-borne disease, caused by the Yellow fever virus (YFV). Historically, the disease originated in Africa and was introduced into South America during the transatlantic slave trade. It is classified as a Group A notifiable disease in Sri Lanka, and it is vulnerable to epidemics due to the presence of its principal mosquito vector, *Aedes aegypti*. In order to prevent the establishment of local transmission following importation of infected travellers, it is recommended that any traveller aged 9 months and above, who has visited or transited (12 hours) through yellow fever endemic countries, should have a valid certificate of yellow fever vaccination when arriving in Sri Lanka, at least 10 days prior to departure from the endemic country.

Epidemiology and Global Distribution

Yellow fever remains endemic in 45 countries across Africa and Latin America, and out of them, twenty-seven African countries and 13 Latin American countries are classified as high-risk for yellow fever outbreaks. An estimated 67 000–173 000 severe infections and 31 000 – 82 000 deaths occur each year in Africa and South America. Although Asia has never reported indigenous transmission, suitable mosquito vectors and favourable environmental conditions create the potential for future outbreaks if the virus is introduced.

Aetiology and Transmission

The causative agent, Yellow fever virus (YFV), belongs to the family **Flaviviridae**, which also includes dengue, Zika, Japanese encephalitis, and West Nile viruses. It is an enveloped, positive-sense RNA virus with a diameter of approximately 40–50 nm, maintained naturally between mosquitoes and non-human primates, and humans become infected primarily through the bite of infected female mosquitoes, predominantly **Aedes** and **Haemagogus** species. Three epidemiological transmission cycles are recognized.



Yellow fever risk classification by country in Africa and South America. Source: WHO

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1. **Sylvatic (Jungle) Cycle**
   In tropical forests, wild mosquitoes transmit the virus among non-human primates. Humans become infected when entering forested environments for occupational or recreational activities.
2. **Intermediate (Savannah) Cycle**
   This cycle occurs mainly in Africa, where semi-domestic mosquitoes infect both monkeys and humans. It commonly results in localized outbreaks affecting rural communities.
3. **Urban Cycle**
   Urban transmission begins when an infected traveller introduces the virus into densely populated areas where *Aedes aegypti* mosquitoes are abundant. Mosquitoes subsequently spread the virus from person to person, potentially causing epidemics.

Clinical Manifestations

The incubation period ranges from 3–6 days. The clinical presentation has two phases;

1. **Initial Phase**
   Most patients develop sudden-onset fever, severe headache, backache, muscle pain, chills, nausea, vomiting and loss of appetite. Many patients recover completely within several days.
2. **Toxic Phase**
   Approximately 15% of patients progress to a severe toxic phase characterized by high fever, jaundice, abdominal pain, persistent vomiting, bleeding from mucosal surfaces, gastrointestinal haemorrhage, acute kidney injury and multi-organ failure. Case fatality among patients entering the toxic phase may reach 50%.

Diagnosis

Clinical diagnosis is challenging because early symptoms resemble several tropical diseases, like Malaria, Dengue, Viral hepatitis, Typhoid fever and Leptospirosis. Laboratory confirmation is done mainly by the detection of Yellow fever-specific IgM antibodies and reverse transcription polymerase chain reaction (RT-PCR).

Management of Yellow fever

There is currently **no specific antiviral therapy** for Yellow fever. Management is supportive and includes fluid and electrolyte management. Early supportive care significantly improves survival.

Prevention

1. Vaccination

Vaccination is the cornerstone of Yellow fever prevention. The live attenuated yellow fever vaccine produces protective immunity in approximately 95% of recipients within one week providing lifelong protection following a single dose. Routine immunization is recommended in endemic countries and for travellers visiting affected regions. In Sri Lanka, Yellow fever vaccination needs to be taken by departing travellers who are aged 9 months and above, visiting yellow fever endemic countries, at least 10 days prior to embarkation. It can be obtained from the Assistant Port Health Office of the Quarantine Unit situated at the Medical Research Institute (MRI), Borella, from 9.00 a.m. to 3.30 p.m. during weekdays. Vaccination is generally contraindicated in infants younger than 9 months, individuals with severe egg allergy, severe immunodeficiency, and certain thymus disorders. Pregnant women should be vaccinated only when the risk of infection outweighs potential risks.

2. Mosquito Control

Vector control remains an essential complementary strategy, particularly during outbreaks. Key interventions include elimination of mosquito breeding sites, larviciding, indoor residual spraying where appropriate and personal protective measures such as repellents and protective clothing.

Surveillance and Outbreak Response

According to international public health guidelines, even a single confirmed case in a non-immune population should trigger an outbreak investigation and immediate public health response, and confirmed cases must also be reported to the World Health Organization under the International Health Regulations (2005). Rapid surveillance is fundamental for preventing widespread transmission, which includes immediate notification of suspected cases, laboratory confirmation, active case investigation, contact tracing, emergency vaccination campaigns and continuous vector surveillance.

Yellow fever is classified as a **Group A notifiable disease** in Sri Lanka, and any suspected case requires immediate notification to relevant public health authorities, including the head of the institute, Medical Officer of Health of that area, Regional Epidemiologist, Regional Director of Health Services, Epidemiology Unit, Deputy Director General (Public Health) and the Director General of Health Services as quickly as possible using telephone, telegram, fax or email by the treating medical officer. Surveillance case definition is "A disease characterized by acute onset of fever followed by jaundice within 2 weeks of onset of first symptoms. Haemorrhagic manifestations and signs of renal failure may occur. And a history of travel to a yellow fever-affected area within the last six days". Although no indigenous cases have been reported in Sri Lanka, continued vigilance is necessary due to the widespread presence of *Aedes* mosquitoes and increasing international travel.

Conclusion

Yellow fever continues to pose a significant threat to global public health despite the availability of a highly effective vaccine. Expansion of international travel, urbanization, environmental changes, and widespread distribution of competent mosquito vectors continue to increase the risk of disease spread. Early detection, robust surveillance, prompt outbreak response, and sustained vaccination programs are critical for preventing epidemics. For countries such as Sri Lanka, maintaining preparedness is particularly important given the widespread presence of *Aedes* mosquitoes and increasing international travel.

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References:

- World Health Organization. (2026). *Rapid risk assessment, acute event of potential public health concern: Yellow fever - Global* (v1). WHO Health Emergencies Programme.
- World Health Organization. (2024). *Yellow fever* [Fact sheet]. https://www.who.int/news-room/fact-sheets/detail/yellow-fever
- Centers for Disease Control and Prevention. (n.d.). *Yellow fever clinical overview.* U.S. Department of Health and Human Services. https://www.cdc.gov/yellow-fever/site.html#hcp
- Quarantine Unit of Ministry of Health Sri Lanka. (2021). *Quarantine services and border health security.* https://quarantine.health.gov.lk/

Table 1: Distribution of Notified Diseases reported by Medical Officers of Health 17th – 23rd Aug 2026 (34th Week)

RDHS	Dengue Fever		Dysentery		Encephalitis		En. Fever		F. Poison-		Leptospirosis		Typhus		Viral Hep.		H. Rabies		Chickenpox		Meningitis		Leishman.		Tuberculosis		Leprosy		WRCD	
	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	T*	C**
Colombo	466	18930	1	18	0	2	0	4	0	21	6	231	0	2	0	15	0	0	7	392	1	80	0	4	42	1246	4	117	85	77
Gampaha	48	15843	0	33	0	34	0	0	0	24	3	365	0	6	0	9	0	0	2	594	2	214	0	39	20	692	0	53	0	24
Kalutara	113	7553	0	34	0	3	0	10	0	62	1	292	0	9	0	17	0	0	8	622	4	102	0	13	12	378	2	65	11	42
Kandy	10	6537	0	40	0	9	0	7	0	38	0	145	0	27	0	19	0	0	1	493	0	101	0	43	16	348	0	19	4	7
Matale	11	2201	0	19	0	5	0	2	0	7	2	153	0	3	0	8	0	0	0	150	0	65	2	366	3	126	0	25	32	34
Nuwara Eliya	4	1056	0	37	0	5	0	3	0	16	1	216	0	40	0	14	0	0	0	353	0	151	0	2	3	175	0	9	13	13
Galle	64	6047	1	29	0	11	0	8	0	59	3	408	0	32	0	22	0	0	6	757	0	131	0	2	11	295	1	44	37	60
Hambantota	51	2739	2	45	0	2	0	0	2	42	8	198	0	26	0	12	0	0	3	168	1	54	8	364	1	95	0	22	52	100
Matara	49	6414	1	20	0	4	0	2	0	11	1	284	0	17	0	17	0	0	3	397	2	139	1	107	0	105	2	25	51	61
Jaffna	20	1031	2	68	0	6	0	19	0	95	0	55	2	242	0	1	0	0	3	284	2	41	0	1	5	126	1	12	99	100
Kilinochchi	2	646	0	12	0	1	0	8	0	1	0	76	0	12	0	3	0	1	0	96	0	14	0	2	2	23	0	2	53	60
Mannar	2	535	0	1	0	3	0	0	0	3	0	45	0	4	0	3	0	0	0	64	0	20	0	5	3	36	1	5	100	100
Vavuniya	4	337	1	21	0	4	0	2	0	8	0	102	0	4	0	1	0	0	0	120	0	34	0	23	2	47	0	7	87	91
Mullaitivu	4	103	0	6	0	2	0	1	0	13	0	35	1	6	0	2	0	0	0	12	0	10	0	10	0	19	0	7	40	40
Batticaloa	20	1531	4	64	1	12	0	2	0	21	0	144	0	2	0	16	0	0	7	245	0	74	0	22	4	108	1	81	82	82
Ampara	12	637	0	53	0	1	0	1	0	16	3	124	0	2	5	16	0	0	7	267	1	63	1	17	2	39	0	19	100	100
Trincomalee	11	746	3	23	0	6	0	2	1	15	1	78	0	8	1	4	0	0	9	184	2	31	0	18	0	102	1	7	100	100
Kurunegala	24	3376	1	22	0	18	0	3	0	66	2	313	0	29	0	13	0	1	5	602	0	293	6	326	6	217	0	51	29	33
Puttalam	28	1853	0	18	0	11	0	0	0	9	3	196	1	25	0	7	0	4	8	126	2	139	0	15	0	111	2	23	56	79
Anuradhapura	22	1082	0	19	0	9	0	3	0	51	1	199	0	26	0	13	0	0	6	431	0	121	5	568	4	184	1	49	53	65
Polonnaruwa	6	816	0	25	0	7	0	2	0	59	0	225	0	7	0	25	0	1	2	379	1	57	4	390	0	60	0	67	18	18
Badulla	1	1678	0	33	0	8	0	3	0	13	0	202	0	41	0	118	0	0	1	333	0	203	1	93	7	172	0	15	9	9
Monaragala	10	1259	0	15	0	3	0	2	0	10	0	223	0	29	3	52	0	1	3	189	1	82	5	169	6	90	0	18	49	57
Ratnapura	58	5241	1	29	0	7	1	7	0	19	13	679	0	31	0	14	0	0	4	288	1	86	0	157	5	280	0	28	74	74
Kegalle	5	2740	0	46	0	6	0	5	0	25	1	311	0	11	0	12	0	0	11	545	0	78	0	13	14	216	0	9	22	22
Kalmunai	1	1020	0	43	0	3	0	1	0	20	0	62	0	1	0	3	0	0	3	375	0	36	0	0	1	84	0	26	6	18
SRILANKA	**1046**	**91951**	**17**	**773**	**1**	**182**	**1**	**97**	**3**	**724**	**49**	**5361**	**4**	**642**	**9**	**436**	**0**	**8**	**99**	**8466**	**20**	**2419**	**33**	**2769**	**169**	**5374**	**16**	**805**	**49**	**56**

Source: WRCD module of the EPINET.
A = Cases reported during the current week;
T*=Timeliness refers to returns received on or before 23rd Aug, 2026.
B = Cumulative cases for the year.
Total number of reporting units 363
C**-Completeness;
C**-Completeness;

Table 2: Selected Vaccine Preventable Diseases & AFP 17th – 23rd Aug 2026 (34th Week)

Disease	No. of Cases by Province									Number of cases during current week in 2026	Number of cases during same week in 2025	Total number of cases to date in 2026	Total number of cases to date in 2025	Difference between the number of cases to date in 2026 & 2025
	W	C	S	N	E	NW	NC	U	Sab					
AFP[1]	00	00	00	00	00	00	00	00	00	00	03	42	43	-2.3%
Diphtheria	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Mumps[2]	00	00	00	01	00	00	00	00	00	01	01	101	104	-2.8%
Measles[3]	00	00	00	00	00	02	01	00	01	03	00	51	02	2450 %
Rubella[3]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
CRS[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Tetanus[2]	00	00	00	00	00	00	00	00	00	00	01	04	04	0 %
Neonatal Tetanus[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Japanese Encephalitis[3]	00	00	00	00	00	00	00	00	00	00	01	00	04	-100 %
Whooping Cough[2]	00	00	00	00	00	00	00	00	00	00	01	20	16	25 %

Key to Table 2

Provinces: **W**: Western, **C**: Central, **S**: Southern, **N**: North, **E**: East, **NC**: North Central, **NW**: North Western, **U**: Uva, **Sab**: Sabaragamuwa.

Data Sources:
Weekly Return of Communicable Diseases: Diphtheria, Mumps, Tetanus, Neonatal Tetanus, Whooping Cough.
Special Surveillance: AFP, Measles, Rubella, CRS.

AFP[1] = No Polio cases
Mumps[2], CRS[2], Tetanus[2], Neonatal Tetanus[2], Whooping Cough[2]—Clinically and/ or laboratory confirmed cases
Measles[3], Rubella[3], Japanese Encephalitis[3]— Laboratory Confirmed cases

AFP—Acute Flaccid Paralysis
CRS = Congenital Rubella Syndrome
NA = Not Available

AFP and all Vaccine Preventable Diseases should be investigated by the MOH Personally.

All patients presenting with fever and a maculopapular rash must be promptly notified and investigated. Blood samples and throat swabs should be collected at the appropriate time window

Comments and contributions for publication in the WER Sri Lanka are welcome. However, the editor reserves the right to accept or reject items for publication. All correspondence should be mailed to The Editor, WER Sri Lanka, Epidemiology Unit, P.O. Box 1567, Colombo or sent by E-mail to **chepid@sltnet.lk.** The Epidemiology Unit should be formally acknowledged in all resulting publications as the primary data source.

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ON STATE SERVICE

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